

- Refills cannot be obtained until 80% of the drug has been used. Next available refill date may be provided; however, the date is an estimate. Cumulative "refill too soon" logic also applies, which looks back at prescription history and considers the amount of medication on hand. Refills for maintenance medications are not considered new prescriptions except when the doctor changes the strength, or the prescription has expired.
- Recurring oral non-specialty and specialty medications must be obtained through the pharmacy benefit. Medications will not be covered when dispensed by other sources, including physician offices, home health agencies and outpatient hospitals.
- Select therapies are included in the Starter Fill Program. For these medications, you will receive 14- or 15- day supplies for the first 2 months of therapy. Your coinsurance will be prorated based on the days of therapy.
- Benefits for certain prescription medications and supplies (e.g., self-injectable or self-administered) are only eligible for coverage when dispensed by a pharmacy, under the pharmacy benefit even if you obtain these medications from your provider.
- Some specialty and non-specialty medications may not be available in a 30-day supply, your coinsurance will be based on days of therapy.
- **A generic equivalent will be dispensed if it is available**, unless your physician specifically requires a name brand. If you receive a name brand drug when a FDA approved generic drug is available, and your physician has not specified Dispense as Written for the name brand drug, you have to pay the difference in cost between the name brand drug and the generic.
- **Why use generic drugs?** Generic drugs are FDA approved prescription medications. They are chemically and therapeutically equivalent to the corresponding brand name drug but are available at a lower price. The FDA requires that generic equivalent medications contain the same active ingredients and be equivalent in strength and dosage to brand name drugs.
- **When you do have to file a claim.** You may need to file a claim for services you received overseas or when another group health plan is primary or if you use a non-Network pharmacy. Visit www.geha.com/Claim for more information on how to file.
- **How to submit a prescription reimbursement claim:**
 - Include original drug receipts and submit to:

CVS Caremark
P.O. Box 52136
Phoenix, AZ 85072-2136

You may also submit prescription reimbursement requests online via Caremark's web portal (geha.com/Prescriptions-Summary) or Caremark mobile app (available for Android and Apple).

Reimbursement will be based on G.E.H.A's costs had the participating pharmacy submitted the claim electronically. Members are responsible for their applicable copayment and/or coinsurance, and the difference between our allowance and the cost of the drug.

- **Our prescription benefit includes a step therapy program.** G.E.H.A's preauthorization process may include a step therapy which requires you to use a generic/preferred medication(s) before a brand and/or non-preferred medication is covered. If you are filling a brand and/or non-preferred medication and have already tried a generic/preferred medication(s), the brand and/or non-preferred medication will be dispensed for the applicable plan copayment/coinsurance.
 - When you try to fill a brand and/or non-preferred medication and you have not tried a generic/preferred medication(s), the pharmacist will contact your physician to notify them of the generic/preferred alternative. If the physician approves, a generic/preferred medication will be dispensed for the applicable plan copayment/coinsurance. If the physician does not approve, a preauthorization review will be initiated to determine the medical necessity of the brand and/or non-preferred drug. If the preauthorization for the brand and/or non-preferred medication is approved, you will be responsible for the applicable plan copayment/coinsurance.
 - Unless there are documented clinical reasons why you cannot take the generic/preferred drug, you may still obtain the brand and/or non-preferred drug, but you will be responsible for 100% of the cost of the brand and/or non-preferred drug, which will not apply to your annual out-of-pocket maximum.
- **Compound Medication:** A compound drug is a medication made by combining, mixing or altering ingredients in response to a prescription, to create a customized drug that is not otherwise commercially available. Some ingredients often found in compounds including, but not limited to, over the counter (OTC) products, experimental or investigational agents, bulk powders, bulk chemicals, and certain bases, are not covered through the prescription benefit. Coverage for other ingredients commonly found in compound prescriptions may also **require preauthorization** before coverage is allowed.